

Chapter 11 (New): The AHS Restructuring Roadmap

Redesigning Vermont's Agency of Human Services for the 2028 Strategic Plan Mandate: A Six-Pillar Framework for System Architects

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Act 167 Community Engagement: Recommendations (2024); AHS Health Care System Transformation Reports (2025); Health Care Delivery Advisory Committee mandate; AHEAD State Agreement (January 2025); Vermont Rural Health Transformation Program Application (November 2025); AHS January 2025 Legislative Presentation; GMCB Act 68 Update (February 2026).

This chapter is written for a specific audience: the AHS staff, Health Care Delivery Advisory Committee members, Vermont legislators, GMCB officers, and hospital and community provider leaders who must translate Vermont's reform legislation into an operational reality. It is simultaneously academic — grounding every recommendation in evidence and analytical framework — and operational — providing the specific organizational design, governance structure, accountability metrics, and implementation timeline that Vermont's transformation requires. The six-pillar framework that organizes this entire book maps directly onto the Statewide Health Care Delivery Strategic Plan that AHS must deliver to the Vermont Legislature by December 2028.

Section 1: The Statutory Mandate — What Act 68 Actually Requires AHS to Do

The most important thing to understand about Act 68's mandate to AHS is that it is not a mandate to write a plan. It is a mandate to restructure a system. The Statewide Health Care Delivery Strategic Plan is the vehicle, not the destination. The destination is a Vermont healthcare system that achieves the five goals of Act 167: reduced inefficiencies, lower costs, improved health outcomes, reduced health inequities, and increased access to essential services. The plan is the document that makes AHS accountable for moving toward those goals.

Act 68 is specific about what the Strategic Plan must contain. Drawing from the statute and the Health Care Delivery Advisory Committee mandate, the plan must address:

Statutory requirement	What it means in practice	Deadline
Statewide Health Care Delivery Strategic Plan	A comprehensive plan for Vermont's healthcare delivery system covering all six dimensions: payment reform trajectory, primary care investment targets, hospital network configuration, behavioral health integration, equity goals, and administrative simplification. Updated every three years.	First delivery: December 1, 2028
Affordability benchmarks	Specific, measurable standards for health care affordability — not aspirational language. Must define what 'affordable' means for Vermont families and how progress toward that standard will be measured.	Established by Health Care Delivery Advisory Committee; incorporated in Strategic Plan
Total cost of care targets	Quantified targets for per-capita healthcare cost growth, aligned with AHEAD Model commitments and Act 68's hospital spending reduction requirements.	Act 68 requires 2.5% hospital spending reduction for FY2026; AHEAD TCOC targets from 2027
Standardized accountability metrics	A defined set of outcome measures — tied to the five Act 167 goals — against which AHS will	AHS already reporting monthly;

	report progress to the legislature monthly and the Strategic Plan will be evaluated.	full metrics framework needed by Strategic Plan delivery
Primary care investment plan	A plan for increasing primary care investment as a percentage of total healthcare spending — an explicit AHEAD requirement — including Blueprint expansion, CCBHC development, and CoCM deployment.	AHEAD requires primary care investment targets from 2027; Strategic Plan formalizes the plan
Health equity strategy	A specific plan for reducing disparities in access and outcomes resulting from demographic factors or health status — required explicitly by Act 68 Goal 1.	Embedded in Strategic Plan; equity measures in monthly AHS reporting
Workforce strategy	A plan for attracting and retaining high-quality healthcare professionals in Vermont — Act 68 Goal 5 — including the use of RHT Program workforce investments.	RHT Program plan covers 2026–2030; must be integrated into Strategic Plan

Figure 11.1 — Act 68 statutory requirements for the Statewide Health Care Delivery Strategic Plan.
Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate (healthcarereform.vermont.gov).

The Strategic Plan is not AHS's alone to write. Act 68 created the Health Care Delivery Advisory Committee — 18 members representing AHS, GMCB, the Health Care Advocate, hospitals, health centers, insurers, health professions, small businesses, and long-term care facilities — to develop and maintain the plan in collaboration with AHS. The committee also consults with the Health Equity Advisory Commission and the Vermont Steering Committee for Comprehensive Primary Health Care. This governance structure means the Strategic Plan must reflect multi-stakeholder input and consensus, not just AHS's internal analysis — a significant operational challenge and an important political constraint.

"Review existing AHS agency structure and program list to identify overlaps and opportunities for efficiency. Align Agency of Human Services agencies with Health Service Areas and meet regularly with hospitals and providers. Fully computerize and integrate Agency of Human Services subunits."

— Oliver Wyman Act 167 Community Engagement: Recommendations, September 2024 — Priority actions for AHS in 2025

Section 2: The Organizational Diagnosis — Why AHS Must Restructure, Not Just Plan

Oliver Wyman's finding about AHS was specific and unambiguous. After 230 meetings with hospital leaders, community organizations, state agency staff, and Vermonters across all 14 Hospital Service Areas, the consultants identified AHS's organizational structure as one of the primary barriers to effective healthcare transformation — not because AHS lacks capable people or good intentions, but because the organization was designed for a different era and a different role.

The current AHS organizational model is what management theorists call a program-silo structure: it is organized around funding streams and program types (Medicaid, mental health, substance use, aging services, developmental services, Blueprint, health care reform) rather than around the populations it serves or the geographic communities those populations live in. This structure made sense when AHS's primary function was administering categorical benefit programs. It does not make sense for an organization

whose primary function has become managing the transformation of an interconnected healthcare delivery system.

The Four Structural Gaps

Oliver Wyman identified four specific structural gaps that AHS's transformation must address. These are not recommendations for improvement — they are diagnoses of organizational failures that are currently impeding transformation.

Gap	What the gap looks like in practice	Consequence for transformation	Required structural change
Program-silo structure misaligned with Hospital Service Areas	A hospital in the Northeast Kingdom that needs to address housing, transportation, behavioral health, and primary care gaps simultaneously must work with separate AHS programs: DVHA for Medicaid, DMH for mental health, ADS for aging services, DAIL for developmental services, Office of Health Care Reform for transformation planning. Each has different administrative requirements, reporting cycles, and funding authorities.	Hospitals and community providers experience AHS as a collection of disconnected programs rather than a coordinated partner. Population health management — which requires crossing program boundaries — is administratively complex to the point of being effectively impossible.	Designate HSA-level coordinators within AHS who convene all relevant AHS programs around each hospital's service area; establish HSA-level governance structures with regular inter-program coordination
Absence of a Planning and Effectiveness function	AHS currently has no dedicated analytical unit that can model the financial and access impact of proposed service site changes, calculate the system-wide cost implications of different regionalization scenarios, or monitor whether transformation investments are producing the intended outcomes.	AHS cannot answer the questions the transformation planning process is supposed to generate: if Springfield Hospital converts to REH status, what is the net impact on healthcare access and system cost in Windsor County? If primary care investment increases by \$X, what is the projected reduction in avoidable ED visits? Without this analytical capacity, strategic planning is opinion-based, not evidence-based.	Establish a Division of Planning, Analytics, and Effectiveness within AHS; partner with GMCB on a joint analytics platform; integrate VHCURES access for AHS planning functions
Under-resourcing for the transformation management function	Act 68 requires AHS to simultaneously: negotiate AHEAD implementation with CMS; manage 14 hospital transformation planning processes; develop the Statewide Strategic Plan; oversee \$195M RHT Program implementation; coordinate with GMCB on RBP and global budget design; and report monthly to the legislature. The Health Care Reform Office has	Transformation activities are sequenced by capacity rather than by strategic priority. The RHT Program contract management alone requires a project manager and contract oversight capacity that does not currently exist. The absence of adequate project management infrastructure is the	Hire dedicated transformation staff using RHT Program implementation funds (explicitly authorized); establish a Project Management Office within the Health Care Reform Office; clarify reporting lines and authority between Health Care Reform and program divisions

	neither the staff nor the organizational authority to manage this portfolio effectively.	single most likely cause of transformation plan failure.	
Fragmented SDOH integration	AHS has programmatic authority over housing supports, food assistance, transportation programs, mental health, substance use, and developmental services — all of the social determinants that Oliver Wyman's systems map showed driving healthcare utilization. But these programs are administered separately, reported separately, and evaluated separately from healthcare outcomes.	Vermont cannot achieve the SDOH-driven utilization reduction that global budgets require if the programs that address SDOH needs are organizationally disconnected from the healthcare delivery system. Every hospital transformation plan that identifies housing or transportation as a utilization driver requires AHS to coordinate across at least three separate program areas to address it.	Establish cross-program SDOH integration protocols; create a shared data architecture that links social service utilization to healthcare outcomes in VHCURES; include SDOH program directors in HSA-level coordination structures

Figure 11.2 — AHS organizational gaps and required structural changes. Sources: Oliver Wyman Act 167 Recommendations (2024); AHS Transformation Reports (2025); Act 68 of 2025.

Section 3: The Redesigned AHS Operating Model — A Population Health System Operator

The organizational model that AHS must become — and that the Statewide Strategic Plan must articulate — is a Population Health System Operator: an agency that manages Vermont's healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically around Hospital Service Areas, and accountable for measurable progress toward the five Act 167 goals.

This is a fundamentally different organizational identity than AHS has historically held. The shift is not cosmetic — it requires changes to organizational structure, staff roles, governance processes, data systems, external relationships, and the agency's own theory of how it creates value. The following sections develop this redesigned model across the three levels at which AHS must operate: statewide system architect, regional network convener, and program delivery operator.

Level 1 — Statewide System Architect

At the statewide level, AHS functions as the architect of Vermont's healthcare delivery system: setting the strategic direction, establishing the payment reform trajectory in partnership with GMCB, allocating capital investments (RHT Program, AHEAD EAST Fund, Act 68 grants), developing the accountability framework, and monitoring system performance against the five Act 167 goals.

The statewide architecture function requires four specific organizational capacities that AHS is currently building but has not yet fully developed:

- **Strategic planning capacity:** The ability to develop, maintain, and update the Statewide Health Care Delivery Strategic Plan on a three-year cycle, incorporating input from the Health Care Delivery Advisory Committee, the Steering Committee for Comprehensive Primary Health Care, the Health Equity Advisory Commission, and other required stakeholders. This is a substantial ongoing function, not a one-time project.
- **Analytics and modeling capacity:** The ability to model the cost, quality, and access implications of proposed system changes before they are implemented — answering questions like 'what happens to access in Windsor County if Springfield Hospital converts to REH status?' and 'what is the projected PMPM cost reduction from the primary care investment plan?' This requires the analytics vendor being procured jointly with GMCB, plus internal staff who can interpret and apply the outputs.
- **Federal relationship management:** Active management of Vermont's relationships with CMS (AHEAD Model), HRSA (RHT Program), CMMI (transformation grants), and other federal partners. These relationships are high-stakes, technically complex, and require dedicated staff who understand both Vermont's specific circumstances and the federal program requirements. The AHEAD State Agreement alone requires ongoing negotiation of methodology, monitoring, and potential withdrawal conditions.
- **Legislative accountability:** The production and delivery of the required monthly transformation reports, the December 2028 Strategic Plan, and the three-year updates. Vermont's legislature has demonstrated active engagement with healthcare transformation — the monthly reporting requirement is a real accountability mechanism, not a formality. AHS must staff this function appropriately.

The Health Care Delivery Advisory Committee: AHS's Governance Partner

Act 68 established an 18-member Health Care Delivery Advisory Committee to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Understanding this committee's composition and mandate is essential for AHS's operational planning, because the committee is not an advisory body in the traditional sense — it has statutory authority to set affordability benchmarks and advise the legislature directly.

Committee composition (18 members):

- AHS Secretary (or designee)
- GMCB Chair (or designee)
- Office of the Health Care Advocate
- Representatives from hospitals, health centers (FQHCs), health insurers
- Representatives from nursing, pharmacy, primary care, and other health professions
- Small business representative
- Long-term care facility representative
- Additional members as specified

Primary responsibilities:

- Setting affordability benchmarks — defining measurable standards for what 'affordable healthcare' means for Vermont families
- Monitoring Vermont's healthcare system performance and its impact on public health
- Collaborating with AHS to develop and maintain the Statewide Strategic Plan
- Incorporating recommendations from the Steering Committee for Comprehensive Primary Health Care
- Advising GMCB on creating a continuous evaluation process for the delivery system
- Providing coordinated recommendations to the General Assembly on healthcare delivery

Operational implication for AHS: The committee requires dedicated staff support — meeting facilitation, research preparation, deliberation management, and recommendation documentation. AHS should treat committee support as a dedicated function, not an add-on to existing Health Care Reform Office responsibilities.

Level 2 — Regional Network Convener (HSA-Level)

At the Hospital Service Area level, AHS must function as a regional network convener — bringing together the hospitals, primary care practices, designated mental health agencies, FQHCs, community organizations, and social service providers within each HSA to develop and execute a coordinated regional transformation strategy.

This is where Oliver Wyman's most important organizational recommendation applies: align AHS sub-units with Hospital Service Areas and meet regularly with hospitals and providers. The mechanism for doing this is the designation of HSA-level AHS coordinators — staff whose primary function is to be the face of AHS within a defined geographic area, convening the relevant AHS programs and community partners around the hospital service area's specific transformation priorities.

Vermont has 14 Hospital Service Areas and a geographically distributed health system. The Blueprint for Health already deploys field staff in each HSA — its Quality Improvement Facilitators, Community Health Team coordinators, and field service staff are embedded in local communities. The Blueprint's HSA infrastructure is the organizational model AHS should replicate and build on for broader transformation coordination.

HSA Region	Key hospitals	Population and demographic profile	Priority transformation challenges
Burlington	UVM Medical Center	~230K, growing, most diverse; Chittenden County non-rural by RHT definition	Administrative overhead reduction at UVMHC; primary care access for growing immigrant population; behavioral health integration at scale
Barre/Montpelier	Central VT Medical Center	~65K, aging, state government center	Radiation therapy and neurology COE development; state employee health plan RBP transition; rural access in Washington County

Rutland	Rutland Regional Medical Center	~62K, aging, lower income	COE development (surgery, neurology, psych); high opioid overdose rates; workforce housing shortage
Burlington-south (White River Jct.)	Mt. Ascutney Hospital	~59K, aging, Dartmouth Health connection	REH or focused scope transition; Dartmouth Health cross-border coordination; rehabilitation COE
St. Albans	NW Medical Center	~53K, rural, Franklin/Grand Isle counties	Cancer surgery and radiation COEs; workforce recruitment; Northern border community needs
Bennington	SW VT Medical Center	~48K, aging, most rural	9 COEs — secondary RSC role; psych services critical; Berkshire/Taconic regional coordination
Middlebury	Porter Medical Center	~46K, aging, Addison County	TBD tier status; Porter affiliated with UVM network; shared service opportunities with UVMMC
Brattleboro	Brattleboro Memorial Hospital	~42K, aging, high SUD/MH burden	Acute general surgery COE; Windham County high suicide/SH rates; Brattleboro Retreat psychiatric coordination
Newport	North Country Hospital	~41K, very rural, Northeast Kingdom	One of most financially vulnerable; REH candidacy assessment; Essex/Orleans county high uninsurance
St. Johnsbury	NE VT Regional Hospital	~39K, very rural, Northeast Kingdom	Cancer surgery and infusion COEs; Caledonia/Essex highest poverty; broadband and telehealth priority
Springfield	Springfield Hospital	~31K, rural, Windsor/Windham border	Memory care and psych-adult COEs; high SUD corridor; transformation plan most complex given financial position
Morrisville	Copley Hospital	~31K, rural, Lamoille/Orleans	Orthopedics and rheumatology COEs; Stowe/ski area seasonal population; workforce housing critical
Randolph	Gifford Medical Center	~28K, very rural, Orange County	TBD tier — among most financially vulnerable; comprehensive assessment needed; AHEAD CAH protections important
Grace Cottage	Grace Cottage Hospital	~14K, very rural, Windham County; 19-bed Vermont's smallest hospital	REH conversion most likely path; coordination with Brattleboro Memorial essential; palliative care specialty worth preserving

Figure 11.3 — Vermont 14-Hospital Service Areas: regional profile and priority transformation challenges for AHS HSA-level coordination. Sources: Oliver Wyman Act 167 Report; Vermont RHT Program Application; AHS Transformation Reports 2025.

Level 3 — Program Delivery Operator

At the program level, AHS continues to administer Vermont's healthcare and social service programs — Medicaid (through DVHA), mental health (through DMH), Blueprint for Health, developmental services, aging services, and others. The structural change is not the elimination of program-level operations but the addition of cross-program coordination mechanisms and the explicit alignment of program operations with the transformation goals and geographic structures established at the statewide and regional levels.

The critical program-level change is the integration of social service programs into the healthcare transformation framework. Oliver Wyman's three imperatives — build housing and fix transportation, pay with RBP and move to global budgets, move care out of hospitals — cannot be achieved by the healthcare programs alone. DVHA, Blueprint, and the Health Care Reform Office can transform payment and care delivery; they cannot build housing or run transportation programs. But the AHS programs that fund housing supports (LIHEAP, general assistance), transportation (Medicaid transportation, EMS), food assistance (SNAP administration), and developmental services are sitting in the same agency — they are simply not coordinated with the healthcare transformation agenda.

The specific structural mechanism for this integration is the HSA Coordinator role: a designated AHS staff member for each HSA region who convenes the relevant program staff from DVHA, DMH, Blueprint, ADS, DAIL, and the Health Care Reform Office around the hospital transformation plan for their region. The HSA Coordinator does not control all program resources — that would require a reorganization far beyond what Act 68 mandates — but they create the coordination layer that currently does not exist.

Section 4: The Six-Pillar Framework Applied to AHS's Restructuring Mandate

The HTR six-pillar framework — Policy, Economics, Technology, Clinical, Equity, and Operations — provides the analytical architecture for AHS's Statewide Strategic Plan. Each pillar corresponds to a domain of AHS's transformation mandate, a set of organizational capabilities AHS must develop, and a set of measurable outcomes AHS must track. The following section maps each pillar explicitly onto AHS's restructuring requirements.

Pillar 1 Policy	AHS function: Legislative compliance, federal relationship management, regulatory strategy
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AHS's policy pillar function is to ensure that Vermont's transformation agenda is legally grounded, federally funded, and legislatively supported. The specific organizational requirements are:

- AHEAD Model management: Active management of Vermont's AHEAD State Agreement, including ongoing CMS negotiation on global budget methodology, monitoring compliance with TCOC targets, and managing the conditions under which Vermont can withdraw if the model is not working. This requires dedicated staff with both healthcare finance expertise and federal program management experience.
- Act 68 compliance tracking: Systematic monitoring of Act 68's statutory requirements — RBP by FY2027, global budgets for non-CAH hospitals by FY2028, Statewide Strategic Plan by December 2028 — and proactive management of the legislative reporting calendar.
- RHT Program administration: Oversight of Vermont's \$195M five-year award, including grant management, contractor oversight, annual reporting to CMS, and alignment of RHT activities with Act 167 and Act 68 goals. This is a major federal grant program requiring professional grant management infrastructure.
- Federal policy monitoring: Tracking changes to Medicare, Medicaid, and federal healthcare policy that affect Vermont's transformation — including Medicaid reconciliation legislation, CMMI model changes, and CMS regulatory guidance — and updating Vermont's strategy in response.

Pillar 2 Economics	AHS function: Payment reform sequencing, financial sustainability monitoring, ROI analysis
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AHS's economics pillar function is to manage Vermont's payment reform transition — coordinating with GMCB on RBP and global budget design, monitoring hospital financial sustainability, and ensuring that payment reform produces the premium reduction and affordability improvement that Act 68 requires.

- Payment reform coordination: Regular coordination with GMCB on RBP methodology development, global budget design, and the premium passthrough monitoring that Act 68 requires. AHS brings the Medicaid perspective and the population health data; GMCB brings the regulatory authority. Neither can execute the economics transformation without the other.
- Hospital financial monitoring: Tracking Vermont hospitals' financial positions against the Oliver Wyman projections — particularly the CAH operating profit gap (\$938 vs. \$2,784 benchmark) and the PPS administrative cost premium (\$2,730 vs. \$1,427 benchmark) — and intervening with technical assistance or grant support before hospitals reach crisis.
- AHEAD EAST Fund management: Directing the up to \$150M annually in additional Medicare funds toward the primary care, behavioral health, home

health, and long-term care investments that make global budgets produce population health improvement rather than just cost reduction.

- Transformation ROI tracking: Measuring whether Vermont's transformation investments — RHT Program, Act 68 grants, AHEAD EAST Fund — are producing the \$400M+ in savings that Oliver Wyman projected. This requires the analytics capability described in the organizational gap analysis.

Pillar 3 Technology	AHS function: Data infrastructure, HIE governance, analytics platform, EHR strategy
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AHS's technology pillar function is to ensure Vermont has the data infrastructure needed to manage a transformed healthcare system. The current data environment — VHCURES as the all-payer claims database, VITL as the health information exchange, the Blueprint clinical data registry, and fragmented program-level data systems — was adequate for monitoring discrete programs. It is not adequate for managing population health at the system level.

- VHCURES expansion and real-time access: AHS and GMCB must expand VHCURES to include more timely claims data, enhance provider connectivity (currently voluntary and incomplete), and make VHCURES data accessible for transformation planning analytics. The November 2025 AHS report acknowledged that most available measures reflect 2023 or earlier data — a two-year lag that makes real-time transformation management impossible.
- HIE strategic development: The Act 167-mandated HIE feasibility assessment — currently evaluating the cost-benefit of a statewide electronic health record — is a technology pillar decision with major implications for the Strategic Plan. A statewide EHR would resolve the fragmented data environment that Oliver Wyman cited as a barrier to coordinated care; the question is whether the investment is justified and whether the governance model is viable.
- Analytics vendor implementation: The joint AHS-GMCB analytics vendor procurement underway must produce a platform capable of modeling transformation scenarios, evaluating hospital proposals, and generating the outcome metrics the Strategic Plan requires. This is the most urgent near-term technology investment.
- Cross-program data integration: AHS's SDOH programs (housing, transportation, food assistance) must be connected to healthcare outcome data so that the relationship between social service investment and healthcare utilization can be measured. Without this integration, AHS cannot demonstrate to the legislature that its SDOH investments are producing healthcare savings.

Pillar 4 Clinical	AHS function: Primary care expansion, behavioral health integration, care model design
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AHS's clinical pillar function is to design and support the care delivery models that Vermont's transformed payment system requires. Payment reform creates the incentive for clinical redesign; AHS must create the programmatic infrastructure that makes clinical redesign possible.

- Blueprint for Health expansion and AHEAD transition: Managing the transition of Blueprint's Medicare payments from the current demonstration to Primary Care AHEAD, expanding PCMH enrollment, and ensuring that the MHI pilot's behavioral health integration is made permanent through AHEAD EAST Fund investment.
- CCBHC network development: Executing the plan to certify five additional CCBHCs by July 2026, covering all major HSA regions, and integrating the CCBHC network with primary care, hospital emergency departments, and the 988 crisis system.

- Care model technical assistance: Supporting hospitals and primary care practices in redesigning their care models for the global budget environment — shifting from volume maximization to population health management, building care coordination capacity, and implementing the team-based care models that primary care transformation requires.
- Long-term care capacity development: Addressing the PACE gap, supporting dementia care COE development, and working with DVHA to expand HCBS capacity so that Vermont's growing elderly population can age in their communities rather than consuming hospital capacity.

Pillar 5 Equity	AHS function: Disparity elimination, geographic access, SDOH programs, culturally competent care
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AHS's equity pillar function is to ensure that Vermont's transformation reduces rather than exacerbates health disparities — particularly the geographic disparities between Burlington's Chittenden County and the Northeast Kingdom, and the access disparities affecting Vermont's Medicaid population, rural residents, and linguistic minorities.

- Geographic equity monitoring: Tracking health outcomes, primary care access, and insurance coverage rates by county — with specific attention to the Northeast Kingdom (6-8% uninsurance vs. 3% statewide) and rural counties with higher rates of behavioral health crisis presentations, opioid overdose, and diabetes.
- Medicaid access protection: Monitoring whether RBP and global budget implementation affects Medicaid patients' access to care — specifically whether the reduction in commercial prices affects hospitals' willingness to maintain Medicaid patient volumes. Act 68 explicitly requires consideration of access protection in the RBP design.
- Culturally competent care investment: Addressing the equity gaps Oliver Wyman documented — lack of language access, cultural mismatch in clinical care, inadequate LGBTQ+ competency, weight bias — through workforce investment (provider diversity recruitment), training requirements, and community partnership programs.
- SDOH program coordination: Ensuring that AHS's SDOH programs — housing supports, transportation, food assistance — are explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization and health disparities.

Pillar 6 Operations	AHS function: Transformation plan management, RHRC successor, administrative simplification
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AHS's operations pillar function is to manage the execution of Vermont's transformation — the 14 hospital transformation plans, the regional coordination processes, the technical assistance programs, and the administrative simplification agenda that Oliver Wyman identified as essential for reducing provider burden.

- Hospital transformation plan oversight: Managing the pipeline from Draft Care Transformation Strategy Outlines through Draft Plans to Final Plans for all 14 hospitals, ensuring plans are coordinated regionally and nationally, and tracking implementation against Act 167 outcome objectives.
- Administrative simplification: Acting on Oliver Wyman's recommendation to further simplify prior authorization, the CON process, and Medicaid billing — reducing the administrative burden that drives physician frustration and consumes provider resources that should go to patient care.
- Project Management Office: Establishing the PMO infrastructure needed to manage Vermont's transformation portfolio — RHT Program, AHEAD implementation, hospital transformation plans, Strategic Plan development —

- with professional project management, risk tracking, and escalation processes.
- EMS and transportation transformation: Coordinating the EMS professionalization and regionalization that Oliver Wyman prioritized — addressing the 31-agency fragmentation that makes inter-facility transfers complex and expensive, and building the transportation infrastructure that population health management requires.

Section 5: The AHS Restructuring Implementation Timeline

The AHS restructuring is not a future event — it is happening now, driven by statutory deadlines that are already creating organizational pressure. The following timeline organizes the key milestones that AHS must meet, grouped by the organizational capability they require.

Deadline	Milestone	Primary owner	Status
FY2026 (now)	Hospital transformation grant awards; all 14 hospitals engaged in transformation planning	AHS Health Care Reform Office	In progress — all eligible hospitals submitted applications
FY2026 (now)	Analytics vendor procurement (joint AHS-GMCB)	AHS + GMCB	Procurement underway as of November 2025
FY2026 (now)	CCBHC certification for 5 new entities (July 2026 target)	AHS Department of Mental Health	On track per RHT application plan
FY2026 (now)	AHEAD Medicaid global budget operational (January 2026)	DVHA / CMS	Operational — Medicaid global budget in effect
FY2026 (now)	RHT Program implementation planning and staff hiring	AHS Health Care Reform Office	\$195M award received December 2025; implementation planning underway
FY2026 Q2-Q4	Final hospital transformation plans (early 2026)	AHS + 14 hospitals + RHRC successor	Plans expected early 2026; AHS integration work follows
FY2026 Q2-Q4	HSA Coordinator roles established	AHS Secretary	Organizational change requiring Secretary-level decision
FY2026 Q2-Q4	Division of Planning and Effectiveness established	AHS Secretary + GMCB Chair	Recommended by OW; critical for Strategic Plan development
FY2027 (Oct 2026)	RBP maximum rates in effect for commercial payers	GMCB (AHS coordination)	Statutory deadline; GMCB leads; AHS coordinates on Medicaid alignment
FY2027 (2027)	AHEAD Model Cohort 2 begins — Medicare FFS global budgets	AHS + DVHA + GMCB + CMS	January 1, 2027; nine-year model; requires ongoing federal coordination
FY2027 (2027)	Primary Care AHEAD operational — enhanced primary care payments begin	DVHA + Blueprint for Health	Medicare primary care payments through AHEAD replaces expiring demo
FY2028 (Oct 2027)	Global hospital budgets for non-CAH hospitals	GMCB (AHS coordination)	Statutory deadline; requires completed RBP methodology and budget design
December 2028	Statewide Health Care Delivery Strategic Plan delivered to legislature	AHS Secretary + Health Care Delivery Advisory Committee	Statutory deadline; updated every three years thereafter

FY2030 (Oct 2029)	Global hospital budgets for all Vermont hospitals including CAHs	GMCB (AHS coordination)	Statutory deadline; completes all-hospital global budget coverage
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Figure 11.4 — AHS restructuring and Vermont transformation implementation timeline. Sources: Act 68 of 2025; AHEAD State Agreement (January 2025); AHS Transformation Reports; Vermont RHT Program Application.

Section 6: A Framework for the 2028 Statewide Health Care Delivery Strategic Plan

The Statewide Health Care Delivery Strategic Plan is the most consequential document AHS will produce in the 2020s. It will define Vermont's healthcare system for the decade following its delivery. The following framework provides the structural architecture for the plan, organized around the six pillars and Act 68's statutory requirements.

Proposed Plan Structure

Section	Content	Length/depth
Executive Summary	Vermont's healthcare crisis in 2028: what changed since 2022; what the system looks like today; the five Act 167 goals and progress toward each; the plan's central commitments for 2028-2031	5-8 pages; accessible to general public and legislators
Section 1: System Context and Diagnosis	Updated version of the Oliver Wyman analysis: Vermont's demographic trajectory, hospital financial positions, payer landscape, workforce status, and SDOH burden as of 2028 baseline. What has improved; what remains urgent.	15-20 pages; includes data tables and visualizations
Section 2: Payment Reform Architecture (Economics Pillar)	Status of RBP implementation; global budget parameters for each hospital; AHEAD TCOC targets and performance to date; total cost of care trajectory 2028-2031; affordability benchmarks and progress	20-25 pages; includes hospital-by-hospital financial data
Section 3: Primary Care and Behavioral Health (Clinical Pillar)	Blueprint enrollment and performance; CCBHC network coverage; CoCM deployment status; Primary Care AHEAD performance; behavioral health access indicators and improvement targets 2028-2031	15-20 pages
Section 4: Hospital Network and Regionalization (Operations Pillar)	Confirmed tier designations for all 14 hospitals; COE network status; transfer infrastructure; shared services implementation; REH/CACC conversions completed or planned; workforce distribution	20-25 pages; includes hospital network map
Section 5: Data and Technology Infrastructure (Technology Pillar)	VHCURES enhancement status; HIE connectivity; statewide EHR decision; analytics platform capabilities; population health data architecture; interoperability with federal systems	10-15 pages
Section 6: Health Equity Strategy (Equity Pillar)	Geographic equity indicators by HSA; disparity measures by income and demographics; SDOH program coordination framework; culturally competent care investments; equity targets 2028-2031	15-20 pages
Section 7: Workforce Strategy	Current workforce supply and demand by profession and geography; RHT Program workforce investments and outcomes; scope-of-practice changes; pipeline programs; 2030 workforce projections	15-20 pages
Section 8: AHS Organizational Architecture (Policy)	AHS organizational structure as redesigned; HSA Coordinator model; Division of Planning and Effectiveness; cross-program SDOH integration;	10-15 pages

and Operations Pillars)	governance relationships with GMCB, DVHA, DMH, DFR	
Section 9: Accountability Framework	Complete metrics dashboard (all 17+ Act 167/Act 68 outcome measures); data sources and update frequencies; performance targets for 2028-2031; reporting calendar; conditions for plan revision	10-15 pages; appendix with full metrics table
Section 10: Financial Plan	Capital allocation plan for RHT Program (remaining 3-year balance); AHEAD EAST Fund allocation; Act 68 grant commitments; projected system savings from transformation; state budget implications	15-20 pages

Figure 11.5 — Proposed structure for Vermont's 2028 Statewide Health Care Delivery Strategic Plan. Sources: Act 68 of 2025; Health Care Delivery Advisory Committee mandate; HTR six-pillar framework.

The Central Commitment the Plan Must Make

Beyond its analytical content, the Strategic Plan must make one central commitment that is currently absent from Vermont's transformation discourse: a specific, quantified trajectory for what Vermont's healthcare system will look like in 2031, 2035, and 2040, with the financial modeling to show it is achievable.

Oliver Wyman provided the diagnostic scenario: without transformation, 13 of 14 hospitals report losses by 2028, premiums reach unsustainable levels, and the healthcare system contracts through unplanned closures rather than orderly redesign. The Strategic Plan must provide the alternative scenario with equal specificity: if Vermont executes the transformation agenda on the timelines described in this chapter, what does the system look like in 2031? How many hospitals maintain inpatient beds? What is the primary care investment level? What is the premium growth rate? What is the uninsurance rate in the Northeast Kingdom?

This is the difference between a strategic plan and a strategic aspiration. Vermont's legislators, hospital leaders, community organizations, and Vermonters deserve to know not just what AHS is doing but what the state will look like if AHS succeeds. The six-pillar framework provides the analytical architecture for making that commitment with intellectual honesty about what is achievable and what remains uncertain.

Statutory deadline: Statewide Strategic Plan delivered to legislature Dec 2028 <i>Act 68 of 2025 — updated every three years thereafter</i>	Health Care Delivery Advisory Committee: members 18 <i>AHS, GMCB, HCA, hospitals, health centers, insurers, professions, small business, LTC</i>
RHT Program capital available over 5 years for transformation \$195M <i>Vermont's December 2025 award; aligned with Act 167 and Act 68 goals</i>	Oliver Wyman projected direct savings from transformation (5 years) >\$400M <i>Independent of AHEAD and RBP payment reform savings</i>

Figure 11.6 — Key parameters for AHS's Strategic Plan development. Sources: Act 68; VTDigger; Oliver Wyman Act 167 Report; Vermont RHT Program Application.

Section 7: Vermont as National Template — Why This Restructuring Matters Beyond Vermont

Vermont's AHS restructuring is not a local administrative exercise. It is the first attempt by a state agency to systematically redesign itself as a population health system operator in response to a statutory mandate for healthcare transformation. Every state

that faces Vermont's demographic and fiscal trajectory — which is to say, every state in the Northeast, most of the Midwest, and eventually every state — will need to answer the same organizational design questions Vermont is answering now.

The questions are not Vermont-specific. How do you align a state human services agency around geographic service areas rather than program silos? How do you build the analytics capacity to model the impact of structural change before you implement it? How do you integrate social service programs with healthcare delivery reform so that SDOH investments actually reduce healthcare utilization? How do you manage a multi-stakeholder strategic planning process that includes hospitals, insurers, providers, and community organizations without producing a lowest-common-denominator plan that satisfies everyone and commits to nothing?

Vermont will have answers to these questions by December 2028 — answers grounded in real experience with a real system facing real pressures. Those answers will be the most valuable policy contribution Vermont has made to American healthcare in a generation. The national significance of Vermont's transformation is not that Vermont is large or economically powerful. It is that Vermont is early — early enough to produce a tested model before the rest of the country reaches the same crisis point, and early enough for the lessons to still be actionable.

This is why the book you are reading exists: to document Vermont's experience in a form that is analytically rigorous, practically useful, and transferable to the healthcare leaders and policymakers who will face Vermont's challenges in their own states and systems. The AHS restructuring chapter is not the end of the story — it is the moment when the story becomes fully operational.

Key Concepts Introduced in This Chapter

Term	Definition
Population health system operator	The organizational identity AHS must adopt — managing Vermont's healthcare and social services ecosystem as an integrated system oriented around population health outcomes, organized geographically, and accountable for measurable progress toward the five Act 167 goals.
HSA Coordinator	A proposed AHS organizational role: a designated staff member for each Hospital Service Area who convenes the relevant AHS programs and community partners around the hospital's transformation plan, creating the cross-program coordination layer that currently does not exist.
Division of Planning and Effectiveness	A proposed new organizational unit within AHS (also recommended as an addition to GMCB in Oliver Wyman's report) to provide the analytical capacity for modeling transformation scenarios, evaluating hospital proposals, and monitoring outcome metrics.
Statewide Health Care Delivery Strategic Plan	The plan AHS is required by Act 68 to deliver to the Vermont legislature by December 1, 2028, and update every three years. The plan must address Vermont's healthcare delivery system across all dimensions: payment reform, primary care, hospital network, behavioral health, equity, workforce, and AHS organizational architecture.
Health Care Delivery Advisory Committee	The 18-member committee established by Act 68 to develop and maintain the Statewide Strategic Plan in collaboration with AHS. Has statutory authority to set affordability benchmarks and advise the legislature directly on healthcare delivery.

Program-silo structure	AHS's current organizational model, organized around funding streams and program types rather than geographic service areas or population segments. Oliver Wyman identified this as a primary barrier to coordinated transformation.
Cross-program SDOH integration	The organizational mechanism for connecting AHS's social service programs — housing, transportation, food assistance — with healthcare transformation goals, so that SDOH investments can be explicitly directed toward the populations and communities where SDOH gaps are most driving healthcare utilization.
Reform cascade	The legislative sequence — Act 167 (2022) → Act 51 (2023) → Act 68 (2025) — in which each intervention builds on the evidence and institutional capacity created by its predecessor. Vermont's AHS restructuring is the organizational expression of this legislative cascade.

Sources: Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024); Vermont Agency of Human Services Health Care Delivery Advisory Committee page (healthcarereform.vermont.gov); AHS Health Care System Transformation Reports (August 2025, November 2025); Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, House Health Care Committee (January 31, 2025); AHEAD State Agreement (January 17, 2025); Vermont Rural Health Transformation Program Application (November 3, 2025); GMCB Act 68 Update on RBP and Global Budgets (February 17, 2026); All prior chapters of this book.